

Revenue OS — Discovery Audit

Clinic name

Meeting date

Pre-call snapshot

Mapped before walking in.

Instagram

WhatsApp response

Paid ads

Review themes

Positioning

Source

ICP Score

/ 10

Automation Opportunity

/ 10

Sales-cycle type

STEP 1

Where the funnel leaks

Funnel Diagnostic — mark each stage from the pre-call audit + what's confirmed live. Evidence = quote or observation, never interpretation.

#	Stage	Status	Evidence
01	Instant Response		
02	Concern Qualification		
03	Lead Scoring (Hot/Warm/Cold)		
04	Booking Automation		
05	Appointment Reminders		
06	No-show Recovery		
07	Post-consult Follow-up		
08	Quote / Treatment Follow-up		

STEP 2

What patients are actually complaining about

Review-Sourced Pain Signals — recurring complaint themes only. Tells you system problem vs. staffing problem. Check once confirmed in the room.

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STEP 3

What to actually pitch

Recommended Modules — only say these out loud after the matching row in Step 1 has been confirmed by them, out loud, in the room.

1. Instant WhatsApp Response Layer **PRIMARY**

Why this, for this clinic:

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2. Quote / Consult Follow-up Sequence **PRIMARY**

Why this, for this clinic:

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3. Booking + Reminder Automation **PRIMARY**

Why this, for this clinic:

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4. Consultation Recovery System **SECONDARY**

Why this, for this clinic:

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5. Dormant Lead Reactivation **SECONDARY**

Why this, for this clinic:

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STEP 4

Capture the system config

Once a module is confirmed, collect what the pipeline actually needs to build it for this clinic. Ask this right after Step 3.

"Assuming we move forward — I want to understand what your system can already tell us, so we're not guessing when we build this."

CRM/booking system in use

What platform do you actually run enquiries and bookings through today?

Services to enable

Confirmed from Step 3 modules

Lead fields captured today

Do you log source, owner, value, intent per lead?

Event-level logging

Is every touch logged with its own timestamp, or only the latest status shown?

Which moments get logged

Lead created -> assigned -> notified -> first contact -> booked -> reminder -> show/no-show -> follow-up -> won/lost — which produce a record today?

Their vocabulary

Their exact stage names, outcome labels, channel labels, intent labels

No-show specifics (if confirmed)

Grace period; reminder schedule; recovery attempts; realistic show->close rate

Deal tiers

Real Rs cutoffs for highest-value vs. everyday enquiry

Business hours & channel urgency

Working hours/days; which channels need fastest response

Staffing structure

How many people handle enquiries, who owns which channel?

Prior automation/CRM attempts

Tried before? What happened, why did it stop?

Peak load / seasonality

Times enquiries spike — campaigns, weekends, season?

STEP 5

Confirm before you scope

Discovery Questions for the Room — ask only the ones relevant to what's still unconfirmed. Log the answer the moment they give it.

What happens today when someone messages after hours — does anything reply before a human does?

Once someone reaches WhatsApp from your other channels, is follow-up entirely manual, or is there a structured system?

How do you currently track a lead that went quiet after a quote or consultation?

Roughly what % of booked consultations are no-shows or late cancellations?

Is there a CRM today, or is enquiry-to-booking tracked manually / in someone's head?

Who owns follow-up when the front desk is busy with in-clinic patients?

Have you noticed any recurring feedback in reviews or from patients — is that something you're already aware of?

For multi-visit treatments, what brings a patient back for visit 2 and 3 today?

+ clinic-specific question:

In the room — Talk Track

Not a script to read verbatim — the shape to keep: confirm before you diagnose, diagnose before you pitch, their numbers before your price.

Open

"Before we get into anything on our side, I want to spend the first part understanding how enquiries actually move through the clinic today — from a DM or ad click to someone in the chair. Is that alright?"

Frame it as mapping, not pitching.

Confirm what you observed

"We went through your funnel ourselves before this call. [Name what worked.] On [channel], we messaged around [time] and didn't hear back until [time/never]. Is that typical, or did we catch an odd day?"

Wait for the real answer — this is where Evidence gets written down.

Tie to their own reviews

"A few reviews mention [theme]. Does that track, or is it outdated?"

Neutral tone — their patients said this publicly, you're just using it.

Ask the open discovery questions

"When a patient doesn't book on the spot — say they want to think about a package — how do you currently follow up?"

Only ask what's still unconfirmed after your pre-call research.

Anchor with their numbers

"Roughly how many enquiries come in monthly across all your channels?" -> "If even [X]% go quiet after the first message, at your average ticket size, that's real revenue you've already paid ad spend for."

Use their number, not a generic industry stat. Keep this directional — don't harden it into a promised figure.

Name the fix — only now

"Based on what you've told me — [confirmed gap 1], [confirmed gap 2] — I'd prioritize [wedge] first. Everything else matters, but it's secondary."

Every module named must map to a row they confirmed above.

Close with a concrete next step

"I'll put together a short written version of what we discussed — the gaps and a proposed first phase — by [day]. Want a rough cost and timeline included, or should we scope that on a second call?"

Gets them to state how they want to be sold to, instead of you guessing.

Reference — Note-Taking Rules

- **Evidence, not interpretation**

"Founder said 'we call back when we can'" — not "follow-up is disorganized." Write what was said.

- **Live, not after**

Fill Status + Evidence as they answer. Reconstructing from memory after the call loses accuracy.

- **Config answers are facts, not estimates**

Step 4 answers should be exact (what the system does today), not rounded guesses. A blank answer means the client doesn't capture that today — flag it as a gap to fix before go-live, don't assume a default.